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DISCUSSION

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Cass informed psychotherapy for gender distressed youth

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ABSTRACT

In April 2024, The Cass Review, an independent review of gender identity services for children and young people, was published in the UK. The Review concluded that there was not enough evidence to justify the UK National Health Service's (NHS) continued routine use of Gender Affirmative Medical Treatments (GAMT) for children and adolescents experiencing gender-related distress. Instead, The Review recommended psychosocial and therapeutic interventions. In recent decades, the role of psychology and psychotherapy in gender care has been predominantly focused on supporting GAMT. The approach recommended by Cass will therefore require NHS therapists to change direction. This paper will outline a therapeutic stance that incorporates the findings and recommendations of The Cass Review, allowing all psychotherapists to start a process of becoming both evidence-informed and culturally competent for working with gender-distressed children.

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Introduction

In response to tensions surrounding the use of medical affirmation with gender-questioning children in the UK, the National Health Service England (NHSE) analysed the evidence for the use of Gender Affirmative Medical Treatments (GAMT) with children (National Institute for Health and Care Excellence, 2020a, National Institute for Health and Care Excellence, 2020b). Two reviews conducted by the National Institute for Health and Care Evidence in 2020 examined the evidence base for puberty blockers and cross-sex hormones and concluded it was 'inconclusive'. As a result, the paediatrician Dr Hilary Cass was commissioned to conduct a full independent review of gender identity services for children and young people, to ensure that the

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care the NHS was offering to these children was safe, effective and evidence based. The Cass Review took almost 4 years, and utilized systematic evidence reviews alongside consultations with clinicians, patients, and advocacy groups. It concluded in April 2024 with 32 recommendations for service improvements (Cass, 2024).

The Cass Review constitutes one of the most comprehensive and methodologically rigorous evaluations undertaken in this field to date, incorporating systematic evidence appraisal alongside clinical practice assessment. The Review concluded that young people with gender-related distress had been 'exceptionalised' compared to others with similarly complex issues (Cass, 2024, p. 12); no other DSM diagnosis leads to such significant and invasive physical treatment pathways, that aim to alter the appearance of the body, starting in childhood (American Psychiatric Association, 2013). The Review found insufficient evidence to support the NHS's continued and routine use of GAMT with young people. It recommended psychosocial interventions as the primary treatment for gender-distressed children, unless stronger evidence of GAMT's safety and effectiveness becomes available (Cass, 2024, p. 30).

Some clinicians told The Cass Review that they felt unprepared for working with gender distress in young people (Cass, 2024, p. 15). Nonetheless, as the numbers of young people exploring their gender has increased, Cass has argued that finding ways to help them has become 'everyone's business' (Cass, 2022). NHSE is now using the Cass Review recommendations as a blueprint for redesigning specialist services, yet many practitioners in non-specialist gender services are also tasked with integrating its conclusions into their work.

Although the Cass Review was not specifically aimed at psychotherapists, this paper will explore how its recommendations, findings and the implicit values inherent within it can inform psychotherapeutic practices when working with gender-questioning young people. Although it will be particularly relevant for psychotherapists in the UK, these principles hold significance on an international level. It describes a non-stigmatizing, inclusive approach designed to assist psychotherapists working with gender-distressed children to reconnect to mainstream developmental thinking. The paper emphasizes the necessity of a holistic care model that considers all facets of a child's lived experience through comprehensive assessments encompassing mental health, neurodevelopment, and social contexts. The paper offers pragmatic recommendations for psychotherapists, focusing on evidence-based interventions for coexisting conditions and stressing the importance of individually tailored clinical formulations as an essential starting point to any intervention. Finally, it highlights the importance of culturally competent practices and understanding biopsychosocial factors in identity development, ensuring

a compassionate, developmentally informed approach that respects individual gender identities while acknowledging the uncertainties inherent in all identity formation.

A return to developmentally informed psychotherapy

The Cass Review places children and young people at the heart of its recommendations. It evaluates existing treatments and approaches as it would for any other presentation in childhood. In this way, it is inclusive and non-stigmatizing, bringing gender distressed children back into mainstream thinking about child development and safeguarding. The Cass Review implicitly acknowledges that, above all else, a child is a child. A trans identified or gender distressed child does not differ from any other child in terms of their broad developmental, emotional or cognitive capacities, hopes or needs. The Cass Review explicitly invites professionals to reclaim and reconnect to what they already know about childhood, adolescence, cognitive function and identity development (Cass, 2024, p. 15).

Key components of Cass informed psychotherapy: A pragmatic therapeutic stance

A therapeutic stance refers to the attitude, approach or posture that a therapist adopts in their interactions with clients, independent of the model or techniques being applied. It encompasses the therapist's mindset, values and the way they engage with their patients. The key components of this stance will be outlined in this paper.

The Cass Review also outlines some pragmatic recommendations for NHS gender services that can be incorporated into individualized therapy in the following ways:

Holistic assessment and care

For clinicians working with gender identity there is a need to take young peoples expressed and felt identity very seriously. However, the term 'diagnostic overshadowing' refers to a cognitive (thinking) bias that might impact a clinician's diagnosis and treatment recommendations where a comorbid condition is overlooked in the face of a more salient one (Hallyburton, 2022). The Cass Review described how most children expressing distress about their gender have more than one problem, but that historically services had been 'dealing with each issue in isolation, without considering the impact of issues on each other' (Cass, 2024, p. 151). To counter this, Cass recommends a model of care that considers all needs together and holistically.

Cass recommended that clinicians should use a framework to ensure a fully holistic assessment takes place, from which an individualized care plan can be developed (Cass, 2024, p. 139). The assessment should include screening for neurodevelopmental conditions and a full mental health assessment. Cass's holistic assessment framework (HAF) recommends that the following areas should be assessed routinely whenever a child presents with gender-related distress; Family, Developmental History, Education Setting and Social Context, Mental Health, Safeguarding, Sexual Development, Physical Health Needs, and Gender Development and Experiences. Psychotherapists are used to conducting holistic assessments of this nature.

Treatment approaches

Cass recommends that standard evidence-based psychological and psychopharmacological treatments should be used to manage distress associated with any comorbid conditions that a young person might also be experiencing, such as low mood or anxiety (Cass, 2024, p. 31). As noted above, The Review points out the lack of specific evidence for the use of psychotherapy for young people questioning their gender. It implies that there is no reason why respectful trans-diagnostic psychotherapy wouldn't help with distress in this population, as it does with other complex presentations (Jeppesen et al., 2021).

Cass informed psychotherapy will share goals common to all psychotherapeutic approaches with children: reducing distress, supporting developmental milestones, and promoting mental and physical wellbeing and safety (Cass, 2024, p. 28). Depending on the clinical formulation (for a full definition see below) it may also specifically aim to explore a variety of ways to understand gender-related distress, reduce gender-related distress, improve any relationships that have been impacted by gender questioning, and empower everyone to consider a range of ways to help each child. This process may include helping people to calmly weigh up the pros and cons of social or medical transition, if these are available and being sought.

As with all therapy, it is crucial to respect everyone's cultural and personal context and belief system, even if it does not align with the therapist's. Ethical practice in therapy includes grounding interventions in evidence where this is available, seeking supervision to enhance technical skills and self-reflection, and adhering to professional guidelines.

Individualized clinical formulation

The Cass Review recommends using both diagnosis and individualized formulation when working with these children (Cass, 2024, p. 28). Cass reminds us that confirming that a young person meets the criteria for the diagnosis of

‘gender incongruence’ or ‘gender dysphoria’ is of limited clinical utility, since the diagnosis has no explanatory power or predictive validity (Cass, 2024, p. 193). Instead, a clinical formulation uses a synthesis of all available information – including medical diagnoses – to develop a hypothesis or hypotheses about the reasons for a person’s difficulties. A good clinical formulation links theory and practice and is, where possible, constructed collaboratively with patients to guide their treatment plans (Baird et al., 2017; Johnstone, 2013). A Clinical Formulation can identify a variety of difficulties, strengths and coping strategies, and how these might interact in idiosyncratic ways for any individual. Clinical formulation is a core skill of the profession of clinical psychology, and appears in the regulatory requirements for clinical, counselling, educational, health, forensic, occupational, sports and exercise psychologists (Health and Care Professions Council, 2009), and psychiatrists in the UK (Royal College of Psychiatrists, 2017).

Tensions surrounding the use of formulation in relation to gender distress

There have been tensions regarding the use of clinical formulation with gender-distressed youth. Many transgender individuals have faced distress and stigma, and understandably clinicians are keen not to contribute to this any further (Hughto et al., 2020). As a result, some have argued against formulating any gender identity concerns (Smith, 2018), while others, including adults who regret receiving GAMT, question why children are not always presented with alternative formulations before being recommended GAMT (Littman et al., 2024). This creates an apparent dilemma: some in this field deem it unethical to provide a formulation regarding gender identity, while others believe it is unethical not to do so. Psychotherapists often navigate such dilemmas, offering tentative formulations that clients can reject or adapt (Johnstone, 2013; Simms, 2011; Thrower et al., 2024). The Cass Review states that it is necessary to offer clinical formulations to young people with gender-related distress as this is consistent with approaches for adolescents with other complex multi-faceted presentations (Cass, 2024, p. 28).

The Cass Review reminds us that ideally formulations should be both client-led and developmentally informed. Where children are not yet able to understand complex, multifaceted issues or engage in more symbolic thinking, psychotherapists must communicate information in an age-appropriate manner to ‘scaffold’ the process of formulation in development (Manassis, 2014).

Developing a psychotherapeutic formulation should be an ongoing process, integrating the client’s lived experiences with information the therapist brings from their training and clinical experience. Unlike traditional clinical diagnoses, a holistic, evidence-informed formulation can evolve throughout

the therapeutic process, often being updated as therapy progresses ('re-formulation') or serving as the culmination of therapy rather than a one-time event post-assessment (Thew & Krohnert, 2015).

How much any formulation should focus on gender will vary across clients and time; clinicians need to avoid diagnostic overshadowing while also ensuring gender specific aspects of distress are adequately attended to where necessary.

Distinguishing ethical psychotherapy from conversion therapy

Concerns have been raised about whether some psychotherapeutic approaches to gender-related distress might unintentionally act as covert forms of conversion therapy, especially if they pathologize or consistently discourage gender transition with no good reason (Ashley, 2022; Vincent, 2023). Interventions that aim only to delay or restrict transition can create invalidating environments, potentially increasing distress and psychological harm (APA, 2021; British Psychological Society, 2022). Conversely, of course, not taking the time to fully explore the potential risks of medical transition beforehand, risking uninformed decision-making with long-term consequences, can also lead to harm (D'Angelo, 2024; D'Angelo et al., 2021).

The Cass Review recognizes the particular developmental vulnerability of children and young people in this context and calls for distinct ethical approaches that reflect this and their evolving identities. In line with guidance from the British Psychological Society (2018) and National Institute for Health and Care Excellence (2018) for working with children and adolescents, it advocates for developmentally informed, individualized care that both promotes well-being and protects against harm. Psychotherapeutic interventions informed by the Cass Review should therefore facilitate open exploration without imposing predetermined outcomes, whether from an affirmative or non-affirmative stance. The goal is to balance respect for the child's autonomy with ensuring their safety, just as in all other areas of health care. Through doing this, therapeutic practices will align with all ethical standards that reject conversion therapy.

A biopsychosocial approach

The Cass Review states there '... is broad agreement that gender incongruence is a result of a complex interplay between biological, psychological and social factors' (Cass, 2024, p. 121). This way of conceptualizing gender-related distress is in line with how we understand most complex physical and psychological human experiences. The

biopsychosocial approach encourages psychotherapists to think broadly about the range of factors that may have influenced any child's developing relationship with their gender, body, mental health and identity leading to a multitude of potential interventions to help each child.

Gen Z and the society that shaped them

The Cass Review includes a chapter focusing on those who were born between 1995 and 2009, acknowledging that as a cohort these young people have expressed more gender-related distress than any previous generation. The Review states 'Every generation encounters new experiences, advances, technologies, challenges and stressors that have a profound effect on their behaviours, attitudes and beliefs.' It goes on to highlight that this cohort 'have different beliefs about the fluidity and mutability of gender than older generations' (Cass, 2024, p. 106). This chapter suggests that these changing beliefs might be related to other experiences that this group went through whilst growing up; more specifically living through a world-wide recession, the COVID-19 pandemic and being the first generation to 'grow up online.'

The Review discusses how this cohort grew up with access to both international peer networks, via social media, and easy access to sexually explicit and potentially harmful online content. It discusses how the mental health of this cohort appears to be worse than previous generations. The Review questions whether 'potentially dynamic psychosocial or sociocultural factors predominate in a significant proportion' of gender distressed children (Cass, 2024, p. 193). Psychotherapists are experienced in exploring the influences of systems and societies on individuals. The Cass Review alerts us to the need to think and formulate about how wider changes in society may, or may not, have interacted with a young person's developmental of their gender identity or gender-related distress.

A psychologically informed understanding of identity development

The Cass Review reminds us that psychotherapists already have many of the tools necessary for working with gender-related distress; therapists have always engaged with issues relating to identity. Despite this, it is hard to find a shared definition of what identity is. Some therapeutic models conceptualize identity as 'a set of memories, beliefs, values, experiences, relationships, and values that create one's sense of self' (Leary & Tangney, 2013), others talk about identities as 'dominant stories' about the self that selectively privilege some experiences over others (Morgan, 2000), whilst others think of identity as a social 'mask' or 'persona' (Jung, 1953). As psychotherapists whichever model we work with we are likely to be attuned to the potential

for identity development to comprise both internal and external phenomena resulting from both conscious and subconscious processes.

Biologically, it is not possible to be born with a fully formed complex social identity, though we may be born with impulses, desires or predispositions that might increase the likelihood of certain identities developing in certain contexts (Donovan & Nehm, 2020). Psychotherapists understand how social identities are developed, refined or challenged through our relationships with others. Without interaction with others, the human brain won't develop sufficiently to have any coherent identity at all (Crone & van Drunen, 2024). All of us learn who we are via an ongoing process of interaction with the world and other people through which potentially inherent internal impulses, drives and emotions are named and responded to by others, resulting in them either being reinforced or weakened. This process of developing an identity, or perhaps more accurately identities, is by its very nature flexible and relational. It is a necessary part of human development that allows us to construct internal working models of 'self' that help us to navigate the world or context into which we were born.

Identities that have been constantly internally and externally reinforced over many years are likely to become fixed to some extent and will tend to feel authentic and 'right'. They may persist for life. Equally over time certain identities or aspects of identity might no longer function for us, will be less reinforced and might change as a result (Reicher, 2004). In acknowledging that gender-related identities also sometimes change, the Cass Review reminds us that an identity doesn't need to be fixed or innate to be meaningful and deserving of respect.

Working with identities in development

Much work in psychotherapy, irrespective of the approach taken, seeks to develop a longitudinal formulation that facilitates our understanding of how specific beliefs or identities are formed and are then subsequently maintained through cognitive, biological, and social processes. Change can often be disconcerting and resource-intensive for individuals, which is why a therapeutic process itself can be challenging for some.

Psychotherapists observe how young people learn to make sense of themselves through their interactions with their environments. Early in life, children primarily receive significant feedback about their identities from parents and family members, gradually transitioning to greater influence from peers and society during adolescence (Leary & Tangney, 2013).

It is entirely normal for identity to be uncertain and fluid during childhood and adolescence; in fact, the exploration of identity is a defining feature of adolescence. As self-reflective skills develop, young people often explore various identities to discover which resonate with them.

Identity consolidation occurs at the end of a developmental process, not at the start. Some elements of identity formed during adolescence are likely to endure into adulthood, yet neither the therapist nor the client can predict with any certainty which might do so. Excessive rigidity or certainty about identity during childhood can sometimes be a problem or indicate a need for security rather than a definitive ‘truth’ about an individual’s future self, a consideration that might be particularly relevant when working with neurodivergent individuals (Reid et al., 2024). The Cass Review emphasizes that children and young people are going through distinct and crucial stages of development. Adolescents frequently exhibit behaviours and decision-making patterns that, shaped by their neuropsychological development, highlight their inherently immature capacities for self-regulation, impulse control, risk-taking, and perspective-taking (Blakemore, 2018). Consequently, determining how to address identity-related distress in children and adolescents is particularly complex, necessitating a good understanding of developmental processes and enhanced support and safeguarding from the adults around them.

Given the very nature of psychotherapy, therapists influence how people understand and relate to themselves, a responsibility they must approach with care. The Cass Review highlights that it is impossible to predict which young people will develop a lasting transgender identity and cautions that GAMT may also inadvertently shape the evolution of someone’s sexuality or gender identity in unpredictable ways (Cass, 2024, p. 178). Therapists must acknowledge both their potential impact on identity development and their inability to predict a child’s future gender identity. From this position, either confirming or denying someone’s long-term identity trajectory won’t be helpful. Instead, the focus should be on fostering a therapeutic environment that is safe and validating, independent of identity.

Working with uncertainty

In medicine, there is a need to increase certainty about the likely safety and efficacy of any intervention (McCartney, 2023). In psychotherapy, on the other hand, helping people to increase their tolerance of uncertainty has been associated with positive health outcomes (Strout et al., 2018). It is sometimes an explicit goal in Cognitive Behaviour Therapy (Palitz et al., 2019). Systemic therapies encourage clinicians to develop a sense of ‘safe uncertainty’ in their work (Mason, 2022/2022), whilst uncertainty is a primary therapeutic position for many psychoanalysts hoping to listen to each client ‘without memory or desire’ (Bion, 1967/1988). Psychotherapists are therefore well placed to help clients bear, manage and even value the uncertainty inherent both in identity development and the therapeutic process itself.

Working with the body

The Cass Review highlights a rising trend of young people exhibiting 'bodily manifestations of distress,' including eating disorders, tics, and body dysmorphic disorder, over recent decades (Cass, 2024, p. 27). Research shows that physical health can be affected by psychological trauma, and individuals may express their distress through their bodies in both conscious and sub-conscious ways. Young people seeking help for gender-related distress are more likely to have experienced Adverse Childhood Experiences (ACEs) and trauma (Kozłowska et al., 2021). Research indicates that people who have experienced more ACEs are more prone to physical symptoms and poor health (Jones et al., 2019). Therefore, it is essential to understand the complex relationship each child or adolescent may have with their body.

Working with gender-distressed youth involves exploring both the concrete and symbolic aspects of embodiment. Symbolically, therapists need to examine any disconnect between a young person's felt gender identity and their physical reality, how they cope with any resulting dissonance, and the impacts of this on their mental health. They need to remain attuned to the potential for some children to dissociate from their body. On a concrete level, therapists should be mindful of clients' attempts to either neglect or alter their physical appearance. Gender distressed young people may engage in practices such as breast binding, genital tucking, or sourcing hormones online, which may provide psychological relief but risks physical harm. Exploring intentional self-harm may also be necessary. Therapists aligned with the Cass framework must be aware of these risks and consider employing usual therapeutic and safeguarding measures where needed.

Supporting social & medical decisions

The Cass Review points out that only medics can be accountable for medical decisions relating to prescribing (Cass, 2024, p. 193). Most psychotherapists are not medically trained. Nonetheless, due to the dominance of GAMT in recent years, psychotherapists may be working with children, adolescents or families who have already socially or medically transitioned or who are considering whether they should. As noted above, The Cass Review outlines the lack of evidence base for any of these approaches, particularly in the context of unknown consequences or side effects over the longer term. Psychotherapists have a duty of candour to be honest about these facts, though sharing them may need to be done in a sensitive way and over time. It is also important for therapists to acknowledge that despite the lack of medical evidence for GAMT, there are many individuals who report high levels of satisfaction following social or medical transitions.

Acknowledging the tension between the lack of evidence at the group level and compelling positive personal testimonies from some who have undergone GAMT is essential when working with young people experiencing gender-related distress (Oosthoek et al., 2024). Nonetheless, whilst therapists are accustomed to valuing personal testimonies, qualitative, and phenomenological experiences as valid forms of evidence (Wren, 2014). The Cass Review emphasizes that not all forms of evidence are equal when it comes to *medical* decision-making (Cass, 2024, p. 55). Cass informed psychotherapists must understand the differing evidentiary requirements in medicine compared to therapy and be able to discuss these differences with their patients (McCartney, 2023).

Becoming culturally competent

Culturally competent psychotherapists seek to understand their clients' contexts and adapt their therapeutic approaches to ensure that interventions are respectful and relevant. They also engage in ongoing self-reflection to try and recognize their own biases and the impact these may have on the therapy they provide. Given that we are discussing gender, and the demographics of those seeking help, therapists should try to be mindful of any unacknowledged transphobia, homophobia/heteronormativity or sexism that might inadvertently impact their work.

This paper aims to increase psychotherapists cultural competence for working with the cohort of young people discussed in The Cass Review. The following themes will also be relevant:

Knowledge Specific to Gender Related Distress

The Cass Review reminds us that there is a great deal of misinformation relating to this topic online, so therapists need to be discerning about where they get their information (Cass, 2024, p. 20).

Psychotherapists should aim to listen to a wide range of voices, from both transgender and clinical communities, and become informed about areas of agreement and disagreement. This knowledge will enable them to respond effectively to client enquiries and help families appreciate that there is no singular approach for understanding and addressing gender-related distress.

It is important to try and keep abreast of the ever changing – formal and informal – language and terminology used in this area. Thought needs to be given to language use in assessments and beyond, including preferred names and pronouns. Psychotherapists need to use language that both shows respect and effectively engages young clients and their families. Psychotherapists need to understand how their use, or not, of certain language might be experienced by each family member, including the ways that it might demonstrate alignment or disagreement with any

position. For transparency and to aid understanding, the meaning of any language used might need to be explicitly discussed and negotiated with a family early on in an assessment. There can be no one right way to do this as language is constantly evolving and each individual and family context is unique.

All psychotherapists working in this area should understand how the NHS came to routinely offer GAMT to children in the past and why The Cass Review couldn't recommend that this continues. They may be working with people with strong culturally informed beliefs and emotions relating to GAMT itself.

Working with parents of gender distressed children

The Cass Review highlights that gender distress can pose unique challenges to family functioning, especially when family members have differing opinions on how to best support a distressed child (Cass, 2024, p. 143). Psychotherapists should aim to engage respectfully with a diverse range of parental beliefs and concerns. Some parents may assertively pursue GAMT if they believe it is necessary, while others may express significant fears or concerns about the lack of evidence and potential risks associated with such treatments. There is the potential for parents in either position to feel alienated or to disengage if they feel dismissed or unheard by a therapist.

Given that parents and caregivers play a vital role in their child's care decisions, they can benefit from reliable psychoeducation and therapeutic support themselves (Shoval et al., 2022). Clinicians should be equipped to discuss with parents various perspectives on gender-related distress in children; including people's positive experiences of GAMT, the potential risks associated with GAMT, its limited evidence base, the reality that it may or may not yield the anticipated outcomes for any given individual and the lack of professional consensus about its use.

Mental health and young people

The Cass Review states that 'There has been a substantial increase in rates of mental health problems in children and adolescents across the UK over the past decade, with increased anxiety and depression being most evident in teenage girls ...' (Cass, 2024, p. 27). The culturally competent psychotherapist will be familiar with the increase in young people seeking help for their mental health across the west, the debates about why this might be happening, and the potential rise in young people integrating diagnoses into their identity development per se (Foulkes & Andrews, 2023).

Intersectionality: LGBTQ identities and same sex attraction

The Cass Review explained that the group of young people seeking help with gender-related distress are heterogeneous (Cass 2024, p. 134), as such they will not all identify as transgender either now or in the future. The Cass Review also described how many young people seeking help with gender-related distress are same sex attracted, though they may not identify as lesbian, gay, bisexual (LGB) or Queer (Cass, 2024, p. 118).

The culturally competent practitioner needs to understand the potentially complex and nuanced relationships between LGB, Transgender, and Queer identity development. The term 'Queer' can be used to describe either a gender or sexuality minority identity. Psychotherapists should be aware that an LGB identity might be based on someone's same sex physiological attraction, their same sex behaviours and/or their social identity which can be independent of these (Geary et al., 2018; Marino et al., 2023).

There are known relationships between gender non-conformity in childhood and later same sex attraction (Marino et al., 2023; Rieger et al., 2008; Steensma et al., 2013). There might be a relationship between experiencing homophobia and gender identity development (DeLay et al., 2018). A proportion of adults who underwent GAMT but regretted it subsequently came to understand themselves as LGB (Littman, 2021). There are sex-based differences in LGB and T developmental trajectories (Kaltiala-Heino et al., 2018, Martos et al., 2015, Xu et al., 2021).

As with all identity development, a young person who is exploring an LGBTQ identity can move between identity categories as part of that exploration. Some will go on to identify as LGBTQ in adulthood and some will not.

The culturally competent psychotherapist will know that there is not one homogenous LGBTQ identity or community, but many identities and communities each with different beliefs, values and lived experiences in relation to gender-related distress (Blechinger, 2016).

Working with minority stress in development

The Cass Review acknowledges that gender distressed young people are often subjected to 'significant minority stress' due to their non-conformity. The minority stress model is a framework that describes unique stressors experienced by individuals from marginalized groups, particularly those related to sexual orientation and/or gender identity. The model describes how minority individuals can face chronic social stress due to stigma, discrimination, and prejudice, which can lead to negative mental health (Meyer, 2003). Culturally competent psychotherapists must be aware that young people experiencing gender distress may feel different or alienated in a variety of ways (Martin-Storey, 2016). Therapists may need to intervene at

a social level to help these individuals manage any stigma, prejudice, or abuse they encounter and/or to encourage social support that values their differences (Bartlett et al., 2022).

The minority stress model was developed for adults and can be valuable for helping young people to locate problems of difference outside of themselves. However, some researchers have invited caution when applying the minority stress model to children without first considering that they are in a developmental process (Goldbach & Gibbs, 2017). Children can experience minority stress because they are gender non-conforming, identify as LGBTQ, are perceived to be LGBTQ or because they are unsure and trying out different LGBTQ identities. Psychotherapists should be mindful of this and encourage young people to seek social support from a range of places, those supportive of any current or evolving gender or sexual identity but also support networks that are independent of their identity, ensuring continuity of social support even if their identity changes over time.

Safeguarding

The Cass Review highlights that ‘safeguarding issues can be overshadowed or confused when the focus is on gender’ (Cass, 2024, p. 142). Particularly with children, psychotherapists must always adhere to their usual training and established systems and procedures to ensure client safety. This might include making referrals to social services if a family is abusive in any way or for psychiatric assessment if a child is suicidal. There is no context where we would hasten a referral for physical health interventions following a child’s expression of suicidal thoughts and this should be the same for GAMT.

Summary and conclusions

Cass informed psychotherapy is a therapeutic stance that incorporates the findings and recommendations of The Cass Review, guiding psychotherapists to adopt evidence-based and culturally competent approaches when working with gender-distressed children and adolescents. Cultural competence in this context requires a deep understanding of the heterogeneous experiences and challenges faced by this cohort, as well as insight into the clinical and societal narratives that shape our understanding of who they are and how best to help them.

The Cass Review emphasizes that interventions for children must be developmentally informed, asserting that gender-distressed young people deserve the same standard of care as other children. Psychotherapists should therefore approach this group in the same way that they would

any other, utilizing their full training and expertise to inform standard clinical best practice. This process should involve thorough biopsychosocial assessment and the development of individualized clinical formulations to guide interventions that aim to alleviate distress and enhance functioning. Therapists should honour and respect individual identities while embracing the inherent uncertainty of identity development, thereby creating a nurturing environment that allows for a diverse range of outcomes.

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